AVERRA · pitch script

Engineering better movement intelligence. · Pranshu Kumar (slides 1–7) and Pawan Shinde (slides 8–14) · about 11:55 total (1716 words)

Slide	Topic	Speaker	Time	Length
1	Title	Pranshu	0:00–0:40	40 s
2	Problem & why now	Pranshu	0:40–1:35	55 s
3	One company, two products	Pranshu	1:35–2:20	45 s
4	ParkinSense	Pranshu	2:20–3:20	60 s
5	TremoSense	Pranshu	3:20–4:15	55 s
6	Competition	Pranshu	4:15–5:05	50 s
7	MVP & validation	Pranshu	5:05–6:05	60 s
8	Market	Pawan	6:05–6:50	45 s
9	Business model	Pawan	6:50–7:25	35 s
10	Pricing & unit economics	Pawan	7:25–8:25	60 s
11	Financial forecast	Pawan	8:25–9:20	55 s
12	Team & growth	Pawan	9:20–10:05	45 s
13	Funding ask & valuation	Pawan	10:05–11:05	60 s
14	Milestones, risk & close	Pawan	11:05–11:55	50 s

Pranshu speaks about 6:05, Pawan about 5:50. The hand-over line closes slide 7. Click cues: each slide's builds follow the order of the script. The same text is in the PowerPoint speaker notes and in the website's notes panel (press N).

Pranshu · slides 1–7

Slide 1 Title

0:00–0:40 · 40 s

Good morning. I'm Pranshu Kumar, and this is my co-founder Pawan Shinde. Together we're building AVERRA. Our line is: engineering better movement intelligence.

AVERRA is a biomedical technology company. We turn precise movement sensing into products people can use every day, at prices built for India.

We start with two products. ParkinSense measures and helps people understand their movement. TremoSense assists people who already live with tremor.

I'll take you through the problem and what we've built and tested. Pawan will then take you through the business, the numbers and exactly what we're asking for.

Slide 2 Problem & why now

0:40–1:35 · 55 s

Start with the people. About 4.6 lakh Indians over sixty live with Parkinson's, and roughly 12.8 lakh more have essential tremor, though that second figure comes from a single study.

Two things fail them. Their movement is measured for a few minutes in a clinic, then not at all. And at home, a tremor of four to twelve hertz turns a spoon into a spill.

Published research backs this up. A 2021 study calls a single clinic visit "only a snapshot" of symptoms. The statement people with essential tremor agreed with most was: "I am embarrassed by my tremor when I try to eat or drink in public." And in a study from southern India, 73 of 100 Parkinson's caregivers reported severe burden.

Why now? Apple opened a movement API in 2018, and a Parkinson's monitoring app was FDA-cleared in 2022. India made device licensing mandatory in 2023. And our over-sixty population is set to more than double by 2050.

Slide 3 One company, two products

1:35–2:20 · 45 s

So here is AVERRA: one company with two complementary products.

ParkinSense is a general wellness wearable for anyone, young or old. It tracks activity and vitals, and its standout feature is tremor detection.

TremoSense is assistive. It physically stabilises a spoon for someone whose tremor gets in the way of eating.

They serve different people and different buying decisions. What they share is real: the same Nordic microcontroller family, the same six-axis IMU, the same signal-processing skills, and the same team. We are not claiming shared revenue or cost savings beyond that.

Slide 4 ParkinSense

2:20–3:20 · 60 s

ParkinSense works like a fitness band. You wear it on the wrist, it senses your movement and heart, and it sends that to a phone or computer, where our software shows the results on a live dashboard.

It tracks steps, activity, heart rate, HRV and blood oxygen. What makes it different is tremor detection: it measures a tremor score, its frequency and how much of the day it lasts, and it knows when you're resting, so tremor is never counted as steps.

Where are we? Streaming, step counting, tremor detection and low-power sleep are built and bench-tested. Heart rate and the dashboard work but still need validation. The mobile app is in development, and a clinical evaluation with ICMR is in progress, with results pending.

It is a wellness product. It does not diagnose Parkinson's, and we make no clinical claim until the ICMR results are in.

Slide 5 TremoSense

3:20–4:15 · 55 s

TremoSense is for people who already have tremor. An IMU in the handle feeds a Kalman filter that estimates roll and pitch. An LQR controller then drives two servos to counter-rotate the spoon head.

On a motorised tremor simulator at four, six and eight hertz, the current build rejects about 90 percent of tremor. It runs a real-time operating system, samples at 208 hertz and uses higher-torque servos. We'll publish the bench logs alongside the code.

Two people with tremor have tried it so far. That is encouraging, but it is not yet a study.

Slide 6 Competition

4:15–5:05 · 50 s

Competitors, with prices checked this month.

WHOOP costs ₹17,990 a year, a Fitbit Charge 6 about ₹12,999, and an Amazfit Bip 6 ₹8,999. They are excellent at sleep, recovery and battery life. None documents tremor or movement-disorder insight. That's our space, and we have to prove people value it.

For tremor assistance, one GyroGlove costs 5,899 US dollars, about 5.6 lakh rupees, for one hand. Liftware sold for 195 dollars in 2015, and a weighted spoon is cheap but passive.

TremoSense is a spoon, not a glove, and we don't claim it matches either product. The opportunity is an Indian-built active aid priced under 2 percent of one GyroGlove, while being honest that it does far less.

Slide 7 MVP & validation

5:05–6:05 · 60 s

What has the MVP proven? Both products work on the bench. Two people with tremor have tried TremoSense informally, with no pre-set metric. ParkinSense is now in a clinical evaluation with ICMR, with results pending. Our bench tests had no preregistered metric, and I want to be upfront about that.

So we've preregistered the next test now. Twelve TremoSense units go to at least ten people with tremor for a self-feeding task. We continue if the median tremor reduction is at least 50 percent and at least 60 percent would pay.

For ParkinSense, the test is dominant-frequency error within 5 percent of a reference, and zero false steps during tremor. The ICMR results will be added the moment they're available.

Our decision is to persevere, and to validate on people before we make any medical claim. With that, over to Pawan.

Pawan · slides 8–14

Slide 8 Market

6:05–6:50 · 45 s

Thank you, Pranshu. We sized the market bottom-up.

TremoSense: Parkinson's plus essential tremor patients, half with tremor that disrupts eating, about 8.7 lakh people. At the launch price spread over a three-year life, that's ₹275.7 Cr a year.

ParkinSense is for everyone, but we only size its first beachhead, households living with Parkinson's: ₹141.3 Cr. As a general wearable the upside is far larger: India bought 35 million smartwatches in 2024, a separate TAM of ₹10742 Cr. We show it as upside only; it is not in our plan.

Filtering to urban patients under neurologist care gives a SAM of ₹51.9 Cr. Our year-three SOM, built from 30 clinics plus online, is about ₹109 L.

Slide 9 Business model

6:50–7:25 · 35 s

The business model in brief. Clinicians and occupational therapists bring us patients. Families buy the device.

Revenue comes in three steps. First, research kits for labs, which need no medical claim. Then TremoSense as a one-time sale, with a rental option for those who can't pay upfront. Then ParkinSense, as a device with an optional subscription.

We assume no reimbursement. All revenue is out-of-pocket or institutional.

Slide 10 Pricing & unit economics

7:25–8:25 · 60 s

Pricing. Today's prototype parts cost about ₹4,062. Two thousand rupees is a volume target for the electronics, not today's cost.

We modelled three cost levels. At a small batch of a hundred, the unit costs ₹4,830. At ₹9,999, after GST, a clinic margin, warranty and shipping, we keep ₹2,538. That's a 31% contribution margin and 40% gross margin, so every unit is profitable from the first sale.

Once supplier quotes and tooling bring cost down to ₹2,720, we cut the price to ₹8,999 and still keep ₹3,886, or 53%. ₹6,999 would still be viable at that cost; we hold it back until the company is profitable.

CAC is about ₹1,079, and LTV to CAC is 3.3. ParkinSense at ₹7,999 reaches 2.0, positive after acquisition cost, though thinner.

Slide 11 Financial forecast

8:25–9:20 · 55 s

The plan starts in July 2027 and runs three years.

Phase one, the first six months, is about proof: the TremoSense user study, the patent filing and a regulatory opinion. It costs ₹8.8 L.

Phase two launches TremoSense, with first sales in month twelve. In year three we scale the clinic network and launch ParkinSense as a wellness product.

Year-one costs are ₹31.6 L, a gross burn of about ₹2.63 L a month. Year two still loses 10.4 lakh rupees as volume builds.

We reach sustained operating break-even in month 23, May 2029. Year three makes an operating profit of ₹18.9 L on ₹2.1 Cr of cash in, and we exit at about ₹2.9 L profit a month.

Getting there needs ₹48.2 L in total. Phase one alone needs ₹10.5 L, and that number shapes our ask.

Slide 12 Team & growth

9:20–10:05 · 45 s

We are both hardware engineers. Pranshu built ParkinSense and co-built TremoSense; he leads the company and the wearable hardware. I co-built TremoSense and lead control and power electronics. We stay the hardware and test engineers ourselves. Our gap is commercial, so from month 19 we hire a clinic sales lead and customer support, and in month 25 an app and firmware engineer for ParkinSense. The equity split will be set in a founders' agreement.

Growth runs through clinics. From a thousand caregivers reached, we target a quarter trying the device and 40 percent of those buying. These are targets, not data.

Three experiments come first: an in-clinic trial-to-purchase test, support-group referrals, and a ParkinSense wellness beta.

Slide 13 Funding ask & valuation

10:05–11:05 · 60 s

Now the ask, kept separate from the full plan. Year one costs ₹31.6 L and reaching profitability needs ₹48.2 L. Today we raise only for milestone one: ₹10,51,171.

We propose a pre-money valuation of ₹75 L. That's a working target to negotiate, not an established value. Two methods frame it. Cost to duplicate gives a floor of ₹44.75 L: 34 person-months of engineering plus prototypes. A Berkus-style scorecard gives about ₹60 L, credited mainly for two working prototypes. The premium above that reflects two products and a clinical evaluation already under way.

At ₹75 L, post-money is P plus I: ₹85,51,171. The investor owns I over post-money, 12.3%. After a 5 percent ESOP pool, the founders keep 82.7%. Even at the floor, the investor would own 19.0%.

The Startup India grant is applied for, not awarded, and the next round is priced on the user-study result.

Slide 14 Milestones, risk & close

11:05–11:55 · 50 s

Here's what the money buys. About ₹2.7 L keeps both of us full-time for six months, ₹2.5 L covers prototypes, tools and the patent, and ₹1.6 L builds the study units and runs the user study. The patent is filed in month two, study units are built in month three, and in month six the user study decides kill or continue. If we continue, we move through EMC testing and the CDSCO filing to first sales in month twelve, and to break-even in month 23.

Why could this fail? Bench results may not turn into easier eating with real food and real hands. If the study shows less than 30 percent reduction, we stop TremoSense and focus on ParkinSense; if it works but people won't pay, we move to clinic rental.

AVERRA: engineering better movement intelligence. We're asking for ₹10.5 L at a ₹75 L pre-money, 12.3%, to reach a clear yes-or-no on a product that's profitable on every unit. Thank you.